

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°145 /MVE-BDBV/06/10/2026



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HIGHLIGHTS (last 24 hours)

Sixty-three (63) new confirmed cases of Ebola Virus Disease (EVD), including 17 community deaths, have been reported. These cases originated in the provinces of Ituri (33 cases) and North Kivu (30 cases). In addition, 10 deaths were recorded in Ebola Treatment Centers (ETCs).

A total of 523 new samples were analyzed; 63 of these samples tested positive for the virus Bundibugyo, or 12.0% positivity.

ACCUMULATIVE CASE 8,728	ACCUMULATIVE DEATH CONFIRMED 4,205	LETHALITY 48.2%	PATIENTS IN ISOLATION/CTE 1066	ACCUMULATIVE HEALED 2,269	TRACKING RATES CONTACTS (Of the Day) 82.0%
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Affected provinces and affected health zones



7 Provinces affected

Haut-Uélé, Ituri, North Kivu,
South Kivu, Tshopo, Bas-Uélé and
South Ubangi



**64 out of 167
health zones affected
(38.3%)**

Summary of key points from the last 24 hours

As of today, 63 new confirmed cases of Ebola Virus Disease (EVD) including 17 community deaths have been reported in 2 provinces: Ituri (33 cases) and North Kivu (30 cases).

In addition, 10 deaths were recorded in the CTEs (8 in Ituri, 1 in North Kivu and in Tshopo (1), bringing the total number of confirmed deaths for the period concerned to 27.

On the other hand, 10 patients (4 in Ituri, 4 in Haut-Uélé, 2 in North Kivu) were declared cured after obtaining two negative results in the control test for Ebola virus disease.

As of October 6, 2026, the DRC had recorded **8,728** confirmed cases and **4,205** deaths, representing a case fatality rate of **48.2%**. Ituri remained the epicenter of the epidemic, accounting for **74.6 %** of cumulative confirmed cases and **52.4%** of new confirmed cases reported in the last 24 hours.

64 out of 167 health zones (38.3%), spread across seven provinces, have been affected since the start of the epidemic.

Table 1. Distribution of confirmed cases and deaths by affected province, as of October 6, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	33	6513	3007	46.2%	28/36 (77.8%)
North Kivu	30	1785	1021	57.2%	17/34 (50.0%)
Haut-Uélé	0	364	153	42.0%	7/13 (53.8%)
Tshopo	0	51	18	35.3%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas-Uélé	0	10	4	40.0%	3/11 (27.3%)
South Ubangi	0	2	1	50.0%	1/16 (6.3%)
Total	63	8,728	4,205	48.2%	64/167 (38.3%)

Table 2. Distribution of confirmed cases and deaths by province and zone as of October 6, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	6513	3007	46.2%	33	10	8	18
Adja	12	1	8.3%	1			0
Aru	8	6	75.0%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	210	47	22.4%			1	1
Boga	2	2	100.0%				0
Bunia	1790	594	33.2%	7	5	1	6
Damascus	40	9	22.5%				0
Drodoro	15	7	46.7%				0
Fataki	84	31	36.9%	1			0
Gethy	9	5	55.6%				0
Kambala	2	0	0.0%				0
Kilo	38	12	31.6%				0
Komanda	176	107	60.8%	1			0
Lita	295	148	50.2%	1	0	1	1
Logo	14	6	42.9%			1	1
Lolwa	35	9	25.7%	1			0
Mahagi	4	1	25.0%				0
Mambasa	41	17	41.5%	4	1		1
Mandima	198	98	49.5%	3	1		1
Mangala	366	157	42.9%	1	0	0	0
Mongbwalu	710	337	47.5%	3	0	1	1
Nia-Nia	314	175	55.7%	3	0	0	0

Nizi	79 2	286	36.1%	4	3	1	4
Nyankunde	130	37	28.5%				0
Rimba	14	5	35.7%				0
Rwampara	1118	385	34.4%	3		2	2
Tchomia	76	25	32.9%				0
To ventilate	N / A	493	N / A				0
North Kivu	1785	1021	57.2%	30	7	1	8
Blessed	409	268	65.5%	9	4	0	4
Good	23	7	30.4%	3	0	0	0
Butembo	286	180	62.9%	3			0
Goma	1	0	0.0%				0
Kalunguta	63	22	34.9%	2	1	0	1
Katwa	608	373	61.3%	2	0	1	1
Kayna	3	2	66.7%				0
Kyondo	37	20	54.1%	1	0		0
Lubero	23	11	47.8%	1	0	0	0
Mabalako	51	20	39.2%				0
Alimbongo	5	3	60.0%	1	1	0	1
Manguredjipa	13	5	38.5%				0
Masereka	34	11	32.4%	2	0		0
Musienene	140	69	49.3%	2	0	0	0
Mutwanga	20	4	20.0%	3	1	0	1
Oicha	34	16	47.1%	1	0	0	0
Vuhovi	35	10	28.6%				0
Upper Uele	364	153	42.0%	0	0	0	0
Boma	43	19	44.2%				0
Mangbetu	1	1	100.0%				0
Dungu	3	1	33.3%				0
Gombari	102	44	43.1%				0
Isiro	92	43	46.7%				0
Pawa	4	2	50.0%				0
Rungu	119	43	36.1%				0
Tshopo	51	18	35.3%	0	0	1	1
Bafwasende	7	1	14.3%				0
Kabondo	5	1	20.0%				0
Lubunga	1	0	0.0%				0
Makiso - Kisangani	27	11	40.7%				0
Mangobo	8	4	50.0%	0	0	1	1
Tshopo	2	0	0.0%				0
Wanie - Rukula	1	1	100.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	10	4	40.0%	0	0	0	0

Buta	1	1	100.0%				0
Viadana	6	1	16.7%				0
Ganga	3	2	66.7%				0
South Ubangi	2	1	50.0%	0	0	0	0
Bulu	2	1	50.0%				0
Total	8728	4205	48.2%	63	17	10	27

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA: Not Applicable.

Table 3. Status of alerts notified by province for October 6, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	128	5	133	22	5	106	0	27	19
Bas Uélé	165	2	167	13	2	152	0	15	0
Ituri	1,227	80	1,307,141	77	1086	3	218	110	
North Kivu	1,257	63	1,320,167	62	1090	1	229	125	
South Ubangi	42	1	43	1	41	0	2	0	
Tshopo	64	1	65	2	62	0	3	2	
Total General	2,883,152	3,035,346		148	2 537	4	494	256	

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

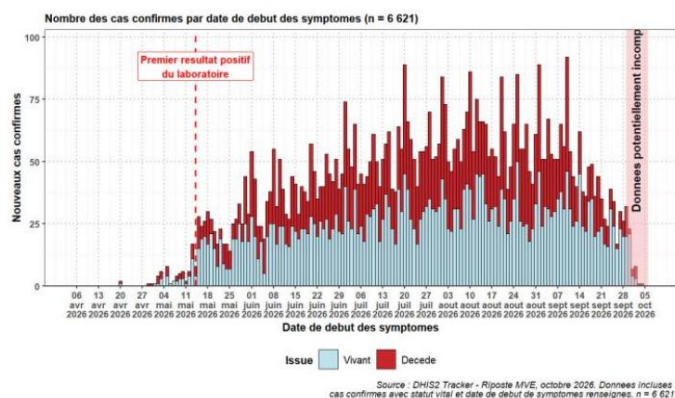
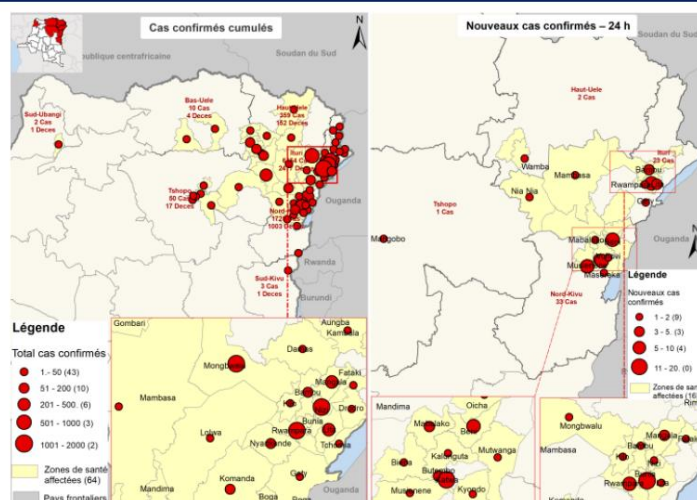


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

- Support for response activities in the affected provinces through the technical pillars.

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.1. Main Actions

On October 6, 2026, **3,035** alerts were reported in the six provinces that had submitted alerts, and all were verified (**100%**) : **494 (16.3%)** were validated as suspected cases and **2,541 (83.7%)** were invalidated. All 494 validated suspected cases were investigated (100%). Of these, **256 (51.8%)** were transferred to COVID-19 treatment centers (Table 3).

Of the **31,832 contacts currently being monitored**, **26,105 were seen in the last 24 hours**, representing a follow-up rate of **82.0 %**, up from the previous day (**80.4 %**), but still below the required threshold of **95.0 %**. This rate was **83.0% (12,569/15,145)** in Ituri, **80.2% (11,452/14,281)** in North Kivu, **87.3% (1,698/1,944)** in Haut-Uélé, **79.8% (225/282)** in Tshopo, **89.7% (122/136)** in Bas-Uélé, and **88.6% (39/44)** in Sud-Ubangi.

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

Two (2) alerts were notified to the PoE/PoC points during the day of October 6th, both in North Kivu, at the Mukulya and Mavivi Barrière PoC points (Beni Health Zone); these were two active alerts, invalidated after verification. In Bas-Uélé, a province not included in Table 4, 11 alerts were notified to the Koteli (7) and Sukisa (4) PoC points in the Koteli Health Zone, all invalidated by the IT department, bringing the national total of alerts notified to PoE/PoC points to 13.

Table 4. Status of key indicators for PoE/PoC by province, as of October 6, 2026

Indicators	Ituri	North Kivu	Tshopo	Overall
Report completeness	96.7% (58/60)	95.2% (20/21)	100.0% (18/18)	97.0% (96/99)
% of functional strategic PoE/PoC	100%	100%	100%	100%
Number of people who have passed PoE/PoC	147,769	92,375	18,763	258,907
% of travelers screened	97.2%	98.0%	98.3%	97.5%
% of travelers who washed their hands	96.8%	98.0%	98.1%	97.3%
% of travelers made aware	99.6%	98.0%	100.0%	99.0%
Number of alerts notified	0	2	0	2
Number of validated live alerts (suspected cases)	0	0	0	0
Number of bodies recovered today	0	0	0	0
% of suspected cases transferred to CTE/CT	N / A	N / A	N / A	N / A
% of lifeless bodies swabbed/secured	N / A	N / A	N / A	N / A
% of suspected cases investigated within 4 hours	N / A	N / A	N / A	N / A

2.2.2. Challenges

In North Kivu, the Butuhe Point of Control (PoC) has been out of operation since the security incident of September 26. 1,837 travelers (2.0%) refused screening, including 511 (11.8%) at the Makeke PoC, and alerts from the PoCs are not being addressed promptly by the alert units. In South Kivu, the completeness of reports is only 55.6% (5/9), and the Kavimvira, Kalundu Port, and Baraka Points of Emergency (PoE) lack any isolation facilities. In Bas-Uélé, only 19 out of 41 notification sites are operational (46.3%), no PoEs have been reinforced (0/11), and the National Humanitarian Action Program (PNHF) services are not operational.

2.3. Laboratory

2.3.1. Main actions

- **Ituri: 33 new positive results** (23 living and 10 deaths) out of 221 samples received and analyzed (**positivity: 14.9%**).
- **North Kivu: 30 new positive results** (23 living and 7 dead) out of 265 samples received and analyzed (**positivity: 11.3%**).
- **Haut-Uélé: 6 new samples** (5 living and 1 dead) all came back negative.
- **Tshopo: 14 new samples (14 blood samples) received and analyzed, all returned negatives**
- **Bas Uélé: 17 samples** (17 blood samples) received and analyzed at the Buta laboratory, **all are negative income.**

2.4. Infection Prevention and Control (IPC/EDS)

2.4.1. Main actions

- **Ituri:** 20 out of 40 rings were opened (50.0%), 44 out of 60 households were decontaminated (73.3%), and 4 out of 8 health facilities were decontaminated (50.0%); no households, social economy enterprises, or schools received kits. Of 100 EDS alerts (including 8 postponements from the previous day), 78 EDS were carried out (78.0%) and 69 bodies were swabbed.
- **Haut-Uélé:** 1 out of 1 health facilities (100%) were decontaminated in Pawa, and 36 service providers were briefed during visits to 10 health facilities in Isiro, Boma Mangbetu, Wamba, and Pawa; no rings could be opened around the 5 targeted cases in Wamba (4) and Boma Mangbetu (1). Of 5 emergency response alerts, 3 were carried out (60.0%), 3 bodies were swabbed, and UNICEF allocated 2 vehicles to the decontamination team in Dungu.
- **Tshopo :** 5 lecture halls and 580 benches at the Faculty of Medicine (case #49), as well as 7 rooms and 2 latrines around case #51 (Tshopo Health Zone), were decontaminated, with the incineration of 2 mattresses, 2 sheets, and 1 garbage can; 18 out of 18 health centers were monitored in 6 health zones, and 186 service providers were briefed (100%). Cumulatively, 40 out of 40 health centers (100%) and 30 out of 33 households (90.9%) that had hosted a confirmed case were decontaminated within 48 hours. Regarding the emergency response teams (ERT), 3 alerts were received (Emergency Treatment Center, Makiso-Kisangani General Referral Hospital, and Rekapi II morgue), 3 bodies were swabbed, and 1 EDT was carried out for 2 bodies at Kibali Gold.
- **Bas-Uélé:** 9 out of 9 ESS (100%) were monitored in Viadana, Buta and Ganga, with briefings for 10 out of 10 providers and 19 out of 19 hygienists (100%); cascade training on IPC began in Viadana for 35 providers from 6 ESS, with support from FHI 360. No EDS was carried out, but 2 swabs were carried out in Poko (AS Kisanga) and in Bili (AS Dumobata).

2.4.2. Challenges

In **Haut-Uélé**, only 4 out of 7 health zones reported their infection prevention and control (IPC) activities, IPC kits for households are lacking, and decontamination efforts around a confirmed case who died on October 5th encountered community resistance. In **Tshopo**, IPC supplies and trained personnel remain insufficient, the incinerator at the Makiso-Kisangani General Referral Hospital is still out of service, and the Rekapi I and II morgues lack sampling kits and personal protective equipment (PPE). In **Bas-Uélé**, the reporting of community death alerts remains low, and 2 out of 3 triage centers monitored in Buta are not operational due to a lack of dedicated staff.

2.5. Vaccination

2.5.1. Main actions

- In **Bas-Uélé**, the vaccination of PPL and TPL continued in the Poko health zone, with the pre-registration of religious leaders, RECO, traditional healers, motorcyclists and hunters, bringing the provincial total to 1,412 people vaccinated out of a target of 13,202 (10.7%), mainly in Buta (620; 21.6%), in Ganga (273; 20.5%) and in Poko (150; 11.0%).
- In **Ituri**, the Partners, Oracle and EBO-PEP clinical trials continued with the support of INRB, Kivu and ALIMA, and a research protocol on the impact of the epidemic on the use of sexual and reproductive health services was received for analysis.

2.5.2. Challenges

In **Ituri**, vaccination of PPL and the study of cross-immunity with the BDBV strain remain suspended pending the supply of Ervebo vaccine by the national PEV.

In **Bas-Uélé**, the implementation of the UCCs is not effective and the validated micro-plan as well as the vaccination funds are still not available.

2.6. Continuity of Care

2.6.1. Main actions

- In **Tshopo**, the evaluation of the City Health Center (Makiso-Kisangani Health Zone) revealed the absence of a maternity ward for more than 3 years, a laboratory and an incinerator, a non-effective triage area, 2 cases of SAM not treated due to lack of inputs (ATPE, F75, F100 and ReSoMal) and only one agent vaccinated out of 18 (5.6%); the recovery plan for the Sendwe Medical Center was developed and the monitoring of PPL at the CTM and the CTE as well as that of recovered patients with comorbidities at the Makiso-Kisangani General Referral Hospital continued.

- In **Haut-Uélé**, a rapid survey on the reasons for the low use of healthcare services has started in the Kolu health district (Isiro health zone), with 28 households surveyed out of approximately one hundred targeted.

2.7. Risk Communication and Community Engagement (RCCE)

2.7.1. Main actions

In **Ituri**, 15,798 home visits reached 77,209 people; 1,078 educational talks reached 11,150 people; 304 mass awareness campaigns reached 8,202 people; 14 community debates reached 691 people; and 12 advocacy sessions involved 44 authorities and leaders. Twenty-four public service announcements were broadcast on 36 radio stations, and six programs were organized. A total of 247 community alerts, including 72 deaths, were reported. 37 out of 55 feedback requests (67.3%) and 1 out of 4 incidents (25.0%) were resolved within 48 hours, and 5 recoveries were reintegrated in Tsere during an awareness campaign that reached 105 people. Only 11 out of 28 health zones (39.3%) submitted their CREC reports.

- In **Bas-Uélé**, 8 mass awareness sessions in schools reached 1,250 people in Ganga, Poko, Likati and Viadana and 132 home visits reached 525 people in Ganga and Viadana; 2 community alerts were notified and resolved.

- In **North Kivu**, 10,654 home visits reached 34,721 people, 230 educational talks reached 7,449 people, and 63 mass communication sessions reached 8,368 people; 201 out of 750 expected community alerts (26.8%) were notified, 13 out of 17 feedback requests (76.5%) were resolved within 48 hours, and 910 out of 4,750 expected participants (19.2%) engaged in community actions. • In **Tshopo**, 7 mass awareness sessions reached 17,466 people in Bafwasende, Tshopo, Mangobo, and Makiso-Kisangani, 202 home visits reached 1,176 people, and 3 educational talks reached 209 people. 57 community alerts were notified, 2 out of 3 refusals were lifted and 186 leaflets and 192 posters were distributed, and the incident related to the attack on a response team on 7th Avenue TP (Kabondo Health Zone) was dealt with.

- In **Haut-Uélé**, 94 educational talks, 7 mass sessions and 212 home visits reached 3,843 people; 43 community alerts, including 5 deaths, were notified and the 15 feedback received was resolved (100%), most of which concerned a perception of patient neglect at the Pawa CTE.

2.8. Mental Health and Psychosocial Support (MHPSS)

2.8.1. Main actions

- In **North Kivu**, the last 30 confirmed cases out of 33 (90.9%), 246 confirmed cases under follow-up, 223 new suspected cases, and 676 suspected cases under follow-up have benefited from SMSPS interventions. Teams assisted 66 households with public health activities, supported 87 people living with HIV, monitored all 8 recoveries in the community (100%), conducted psychoeducation for 304 people (117 men, 146 women, and 41 children) with UNICEF support, and announced 145 results, including 39 positive cases.

- In **Ituri**, 8 of the last 30 cases were confirmed (26.7%), 84 confirmed cases are currently being monitored out of 141 (59.6%), 53 new suspected cases are out of 75 (70.7%), and 105 suspected cases are currently being monitored out of 156 (67.3%).

benefited from SMSPS interventions. The teams supported 275 caregivers and 60 PPLs, followed up with 18 out of 20 recovered individuals (90.0%) in the community and 15 out of 15 children in daycare, supported 16 out of 19 households of separated and orphaned children (84.2%), reintegrated 4 out of 4 recovered individuals, facilitated 2 daycare admissions and 2 family reunifications, and announced 31 outcomes, including 8 positive results.

- **In Tshopo**, the last confirmed case and the 6 confirmed cases under follow-up, as well as 4 new suspects out of 4 and 10 suspects under follow-up out of 10, have been taken care of (100%); the teams supported 9 PPLs and 3 households, followed up on 1 recovered person in the community, lifted 3 out of 3 resistances (Assembly of Saints church, CM Bolila and CM Massimiliano), carried out psychoeducation for 947 people, including 880 students in Lubunga and Kabondo, and announced 4 results, including 1 positive; 5 out of 7 health zones that notified are covered (71.4%).
- **In Haut-Uélé**, 5 recent confirmed cases, 26 confirmed cases under monitoring, 4 new suspected cases and 20 suspected cases under monitoring benefited from SMSPS interventions in the Ebola Treatment Centers of Isiro, Pawa, Boma Mangbetu and Wamba; the teams supported 49 caregivers, 82 PPLs (including 25 members of the national coordination), 10 children separated in the community and 15 household members, supported 3 EDS and announced 37 results, including 8 positive ones
- **In Bas-Uélé**, 5 PPLs, 6 household and community members and 3 contacts were supported, 2 discharged healers are being monitored, 3 results were announced (all negative) and the reintegration of 2 healers from the Viadana health zone was prepared with the CREC pillar).

2.8.2. Challenges

- **In Ituri**, only 14 out of 28 affected health zones are covered (50.0%) and the Mandima health zone has no SMSPS coverage, which requires strengthening the capacities of psychologists and APS/TPS to ensure interventions around cases.
- **In Bas-Uélé**, SMSPS coverage remains embryonic, with activities having only just begun in the Viadana ZS.
- **In Haut-Uélé**, the mobility of community health workers remains difficult in Wamba and Boma Mangbetu, nutritional kits are lacking for weaned infants in Wamba and Isiro, the community health worker lacks computer equipment, and data collection remains weak in Boma Mangbetu and Pawa
- **In Tshopo**, there is a need to train the APS of the ZS of Wanie-Rukula and Bafwasende and a shortage of communication units and motorcycles for the movement of the pillar.

2.9. Logistics

2.9.1. Main actions

- **In Bas-Uélé**, deployment of surveillance teams in Ganga and Viadana for contact tracing, alert verification and active search, support for the registration of SGI members, security and strategic coordination, installation of windows in the DPS meeting room, painting of Hygiene and Education offices with support from WHO and continuation of the construction of the CTE at the HGR in Buta, whose perimeter walls are completed.
- **In Tshopo**, validation of the logistics monitoring checklist, finalization of the needs assessments, monitoring of the construction of the Bafwasende CTE, the extension of the Weather CTE and the isolation of Konga-Konga, affected by the road construction work, and support for the mobility of the PCI, PEC, Communication and PoE/PoC pillars; the mobility needs of the ZS remain estimated at 154 motorcycles.
- **In Ituri**, 15 out of 33 ambulances were mobilized and 25 patients were evacuated to care facilities.
- **In Haut-Uélé**, 6 ambulance trips transported 6 patients to Boma Mangbetu (3), Wamba (2) and Pawa (1), and UNICEF allocated 2 vehicles to the Dungu Health Zone for the decontamination team.

2.10. Security

2.11. Main actions

- **In Tshopo**, the focal point of the Kabondo Health Zone surveillance pillar was seriously injured in the head by machete blows during an investigation, and a response team was attacked by the community on 7th Avenue TP (Kabondo Health Zone) while searching for contacts related to the community death of an escaped MVE
- **In North Kivu**, the Butuhe PoC (Kalunguta Health Zone) remains closed since the security incident of September 26, 2026 which cost the life of a service provider, and the threat persists against the service providers of the PoCs on the Butembo axis.

2.12. PSEA (Prevention of Sexual Exploitation and Abuse)

2.12.1. Main actions

- **In Ituri**, 43 participants (23 women and 20 men), including CTE service providers, day laborers and new Mercy Corps staff, were briefed on the PSEAH and signed the code of conduct in the Bunia Health Zone; 40 motorcyclists, reed sellers and motorcycle repairers (21 men, 9 women, 3 girls and 7 boys) were sensitized, and the posting of messages and the distribution of leaflets continued in Bunia, Aru, Mongbwalu, Mambasa and Fataki; only 13 out of 36 Health Zones (36.1%) know the reporting mechanism.

2.12.2. Challenges

At the national level, only 1 out of 7 DPS (14.3%) reported its PSEA activities; in Ituri, activities do not yet cover all affected health zones, mass awareness materials are lacking and the mapping of stakeholders remains incomplete.

KEY MESSAGES

• In the provinces of Ituri, North Kivu and Haut-Uélé, it is observed active community transmission while sporadic notification is observed in Tshopo province.

2. In the last 24 hours.

• Over the past 24 hours, all suspected cases (100.0%) have been investigated; performance is consistent with data reported by the provinces. However, contact tracing remains inconsistent (82.0% vs. 80.4%) and warrants close attention to prevent uncontrolled chains of transmission.

Maintain a level of vigilance regarding epidemiological surveillance aspects, consolidate IPC and CREC actions at the community level while providing quality care in dedicated care structures with adequate reception capacities.

TIMELINE OF KEY EVENTS

• From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.

• Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.



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